



Armada Kalamunda Group

Vaginal Birth After Caesarean Section Guideline

1. Purpose

The increasing rate of primary caesarean section (CS) has led to an increased proportion of women with a history of previous caesarean birth¹. The options for the next birth include:

A planned Trial of Labour After Previous Caesarean section (TOLAC) which will result in either a vaginal birth or an emergency CS; or

- An elective repeat CS (ERCS)

To provide evidence-based information regarding the care of women undergoing planned Trial of Labour After Previous Caesarean section (TOLAC).

This guideline provides assistance for clinicians to care for and support women:

- In their decision making about their next birth after CS
- Planning a TOLAC

Findings from a large systematic review concluded planned VBAC is a reasonable and safe choice for most women; however, there are individual specific considerations which may increase the potential harms associated with planned VBAC. Facilitating shared decision making will enable women to make informed decisions about their birth options. Women considering their options for birth after a single previous lower segment caesarean should be informed that, overall, the chances of a successful planned VBAC are 72– 76%.

2. Applicability

This guideline applies to medical and midwifery staff working within the Armadale Health Service maternity unit.

3. Overview

All women with a history of one previous LSCS will be referred to the Next Birth After Caesarean (NBAC) clinic for antenatal care and counselling. Women with one previous LSCS who are planning TOLAC must be seen by specialist Obstetrician at least once in their pregnancy before or at 34-36 weeks. Women who have had one vaginal birth pre or post caesarean section can be followed up in GPO clinic and do not necessarily require a specialist clinic appointment.

Counselling will be provided to the woman and her partner by the NBAC midwife or GP Obstetrician/ Specialist Obstetrician, ensuring they are aware of the benefits and risks of TOLAC in comparison to the surgical and anaesthetic risks of Repeat Elective Caesarean Section (ERCS).

3.1 Potential benefits and harms of VBAC and ERCS

VBAC and ERCS have differing risks and benefits for women and their babies. There is insufficient high level evidence in regards to the benefits and harms of planned VBAC and ERCS.

3.1.1 Potential benefits of VBAC

Compared to CS, women having a VBAC have:

- Shorter stays in hospital
- Lower rates of deep vein thrombosis
- Enhanced mother-infant bonding, including the long term wellbeing of the infant
- Lower maternal morbidity
- Infants with a gut microbiota that is causally linked with greater protection from allergic disease

3.1.2 Potential harms of planned VBAC

- Planned VBAC which results in vaginal birth is associated with fewer complications than an ERCS
- Planned VBAC which results in an emergency CS is associated with more complications than an ERCS
- Absolute risk of birth related perinatal loss with planned VBAC is comparable to the risk for women having their first baby

Completion of AKMR70.7“Trial of Labour after Caesarean (TOLAC) checklist are to be signed by midwife and GP and where required, signed by the woman.

If the woman does not wish to have the recommended care a non-standard management sticker must be completed, and the outcome of the consultation fully documented.

4. Guidelines

4.1 Woman-centred care and shared decision making

Woman centred care and shared decision making enables women to feel empowered in making decisions, based on the woman’s individual needs and preferences and the best available evidence of potential benefits and harms, in partnership with care providers³.

Supporting women with their decision making for their next birth after CS includes the principles of:

- Shared decision making, incorporating
 - Clinicians having counselling skills and access to consistent evidence-based information in order to support women to make an informed choice
 - Providing women with consistent evidence-based information about their next birth after CS

4.2 Criteria for suitability for Trial of Labour After Caesarean

- Women with a prior history of one uncomplicated lower segment caesarean section, in an otherwise uncomplicated pregnancy at term, with no contraindication to vaginal birth.

4.3 Contraindications for Trial of Labour After Caesarean

Absolute:

Ascertain and document if contraindications for VBAC are present. Contraindications include:

- Maternal or fetal reasons to avoid vaginal birth in current pregnancy
- Previous uterine incision other than lower transverse segment
- Previous uterine rupture
- Previous hysterotomy or myomectomy entering the uterine cavity

Relative contraindications for TOLAC

- Previous significant sepsis post caesarean section
- Multiple pregnancy requires consultation with specialist Obstetrician
- Known fetal macrosomia (4Kg or more) or fetal intrauterine growth restriction, requires consultation with specialist Obstetrician
- Body mass index (BMI) greater than 30
- Less than 18 months from caesarean birth

4.4 Likelihood of VBAC

Planned VBAC success is generally in the range of 60–80%² Factors which increase vaginal birth occurring include:

- Previous vaginal birth, whether before or following the CS, is a strong predictor^{2,8} with a VBAC rate approaching 90%
- Younger maternal age
- Caucasian/white ethnicity
- Body Mass Index (BMI) less than 30 kg/(m²)
- Weight loss increases VBAC success in women who were overweight or obese before their first CS birth
- Prior CS indication not related to arrest of labour
- Spontaneous onset of labour at less than 41 weeks gestation
- Cervical dilatation greater than 4cm on admission
- Birth weight less than 4 kg

5. Management

5.1 Antenatal Care:

- A thorough obstetric history should be taken and recorded at the initial consultation and documented in the handheld pregnancy record. Relevant information should also be documented on the Obstetric Risk Form (AKMR 70.5). Obtain copy of theatre notes of previous surgery if possible. Special consideration should be given to ultrasound scans for placental location, estimated fetal weight and diabetes screening.
- Counselling will be provided to the woman and her support person by the NBAC midwife or GP or Specialist Obstetrician.
- A NBAC schedule for appointments should be given.

- A 'TOLAC Pack' will be given to the women which includes RANZCOG information on Caesarean sections and Vaginal Birth after Caesarean; AHS Information on Next Birth After Caesarean and AKMR70.7 Trial of Labour after Caesarean (TOLAC) checklist. The AKMR 70.7 form is to be kept in the patient's hospital file.
- A final decision for mode of birth should be agreed between the woman and GP Obstetrician and/or Specialist Obstetrician before the expected/planned delivery date (ideally by 36 weeks of gestation) Consent for TOLAC should be obtained by 36 weeks of gestation.
- Medical Induction of labour with the use of prostaglandins gel is not recommended for TOLAC.
- Mechanical induction with a balloon catheter or ARM is the preferred method.
- Use of Syntocinon post Artificial Rupture of Membranes (ARM), if patient is not contracting should be left to the discretion of the on call team.
- A plan for delivery needs to be made and documented on the Obstetric Risk Form AKMR70.5 and woman's handheld pregnancy record.
- If all parameters for mum and baby are WNL's, women can be allowed to reach term plus 10 days, to maximise their chance of establishing spontaneous labour.

5.2 Intrapartum Care and Monitoring:

- Advise the woman to present to labour ward when contractions are regular and painful, if they have spontaneous rupture of membranes, any vaginal bleeding, or if continuous pain is present.
- An admission cardiotocograph (CTG) should be performed. If the woman is not in established labour the CTG may be removed once a reassuring trace is obtained to allow the woman to ambulate until her labour is established
- Notify the GP Obstetrician and Specialist Obstetrician on call of the woman's admission using the ISOBAR handover technique.
- Review the plan of care prepared antenatally in consultation with the woman and revise if necessary
- Provide one-to-one midwifery care
- Establish Intravenous access with a wide bore cannula (16 g or larger.) Blood should be collected and sent to the laboratory for Full Blood Count and Hold serum.
- The woman may continue to have isotonic non particulate fluid such as Lucozade, isotonic drinks and particle free fruit juices (no milk) once in established labour but should refrain from having solid food.
- Once the woman is in established labour continuous intrapartum fetal monitoring is mandatory for a TOLAC labour. Telemetry is available to aid the woman's mobility.
- 15 minute breaks from continuous fetal monitoring can be considered if the fetal heart rate remains normal and after discussion with the Obstetrician responsible for the woman's care and documented on the CTG and AKMR73.
- Once in established labour the woman's heart rate should be recorded half hourly and documented on the CTG and in the MR55. Use of maternal pulse oximetry is highly recommended in active labour in woman on syntocinon infusion.
- Epidural analgesia is not contraindicated in established labour.
- In addition, to routine assessment and observations, assess:
- Vaginal examination with informed consent:

- Within 1 hour of admission, and then
- Once labour is established:
 - 4 hourly /if indicated until 7 cm dilated, then consider
 - 2 hourly /if indicated after 7 cm dilatation
- Vigilant use of the partogram AKMR72 should be maintained once in active labour, including use of an action / intervention line drawn onto the partogram. The action line becomes the 'intervention' line in a VBAC.
- Delay in progress or any deviation from normal needs to be discussed with the Specialist Obstetrician on call.
- Indications for additional care:
- Augmentation of labour
- If indicated, the use of ARM and/or Oxytocin to augment labour must be discussed with the woman and obstetrician prior to commencement.
- If there is a delay in progress and in the active stage of labour, perform ARM prior to consideration of Oxytocin augmentation
- Oxytocin augmentation is associated with:
 - An increased risk of uterine rupture (2-3 fold as compared to spontaneous TOLAC) and should be used with caution
 - A VBAC rate of 68% (95% CI: 64–72%) (Strength of evidence is low)²
 - A history of previous uterine surgery is a (manufacturer recognised) contraindication:
 - Obtain informed consent and document in the woman's notes
- A senior experienced midwife should care for a woman receiving oxytocin in this situation.
- Oxytocin augmentation should be titrated such that it should not exceed the maximum rate of contractions of three to four in 10 minutes with good relaxation to the baseline in between.
- Careful serial cervical assessments, preferably by the same person, are necessary to show adequate cervical progress, thereby allowing augmentation to continue.
- During oxytocin augmentation the woman's heart rate should be recorded continuously on the CTG, using the pulse oximetry attachment, so that maternal and fetal heartrate can be differentiated immediately.

Observe for signs and symptoms of scar rupture. Uterine rupture may occur at any stage of labour and can occur during pregnancy. There are no reliable clinical markers or models that predict uterine rupture. The signs and symptoms of uterine rupture are typically non-specific, some are rare and some may be associated with other obstetric circumstances, making diagnosis of uterine rupture difficult. Assess in the context of the woman's individual circumstances.

There are no reliable clinical signs or symptoms that predict the timing of uterine rupture. 70% of cases of uterine rupture first present with prolonged fetal bradycardia.

Should any of the following symptoms occur, immediate consultation with the Specialist Obstetrician on call or if not available with any other on site specialist Obstetrician is required:

- Fetal Distress

- Suprapubic tenderness and/or severe constant abdominal pain between contractions.
 - Maternal tachycardia, hypotension, shock
 - Chest or shoulder tip pain, sudden shortness of breath
 - Vaginal Bleeding
 - No progress in labour
 - Cessation of contractions/in-coordinate uterine action
 - Regression of presenting part
 - Haematuria
 - Ensure accurate documentation on the partogram AKMR72 and AKMR55A
- Category 1 Caesarean Section is required for suspected uterine rupture as there is an urgent threat to the woman and her baby¹¹.*

5.2.1 Second stage of labour

Reassess and consult obstetrician if duration exceeds:

- 1 hour for passive descent, and/or
 - 1 hour for the active stage in the woman who has not been in the active stage previously
 - 30 minutes of the active stage in the woman who has previously laboured

When escalating care, e.g., reviewing bloods, USS, or CTG concerns. The reporting midwife **MUST** co sign with the medical officer the acknowledgement of results reviewed and/or any follow up care planned.

6. Postpartum care

Women should be offered the opportunity to discuss the implications for future pregnancies of their birth experience. This discussion may be assisted by an interpreter and/or an Indigenous health worker where appropriate.

7. Legislative context

The following documents are required to give effect to this policy:

- Health Department of Western Australia (HDWA) [Clinical Handover Policy MP 0095](#)
- HDWA [Consent to Treatment Policy OD 0657-16](#)
- East Metropolitan Health Service (EMHS) [Patient Identification and Procedure Matching EMHS: 21](#)

8. Supporting information

The following documents support the implementation of this policy:

- [Recognition and Response to Acute Deterioration Policy AKG-POL-0388-14](#)

9. Compliance, monitoring and evaluation

Compliance with this policy will be monitored via:

- Monitoring of reported clinical incident data via Datix CIMS.

Department specific findings are to be reported at the safety and quality departmental meeting for review and identification of any opportunities for improvement.

10. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 6 – Action 6.5 Correct identification and procedure matching
- The health service organisation:
 - a. Defines approved identifiers for patients according to best-practice guidelines
 - b. Requires at least three approved identifiers on registration and admission; when care, medication, therapy and other services are provided; and when clinical handover, transfer or discharge documentation is generated
- Standard 6 – Action 6.7 Clinical handover
- The health service organisation, in collaboration with clinicians, defines the:
 - a. Minimum information content to be communicated at clinical handover, based on best-practice guidelines
 - b. Risks relevant to the service context and the particular needs of patients, carers and families
 - c. Clinicians who are involved in the clinical handover
- Standard 6 – Action 6.9 Communicating critical information
- Clinicians and multidisciplinary teams use clinical communication processes to effectively communicate critical information, alerts and risks, in a timely way, when they emerge or change to:
 - a. Clinicians who can make decisions about care
 - b. Patients, carers and families, in accordance with the wishes of the patient
- Standard 8 – Action 8.4 Recognising acute deterioration
- The health service organisation has processes for clinicians to detect acute physiological deterioration that require clinicians to:
 - a. Document individualised vital sign monitoring plans
 - b. Monitor patients as required by their individualised monitoring plan
 - c. Graphically document and track changes in agreed observations to detect acute deterioration over time, as appropriate for the patient
- Standard 8 – Action 8.6 Escalating care
- The health service organisation has protocols that specify criteria for escalating care, including:
 - a. Agreed vital sign parameters and other indicators of physiological deterioration
 - b. Agreed indicators of deterioration in mental state
 - c. Agreed parameters and other indicators for calling emergency assistance
 - d. Patient pain or distress that is not able to be managed using available treatment
 - e. Worry or concern in members of the workforce, patients, carers and families about acute deterioration

11. References

1. Australian Commission on Safety and Quality in Health Care. Shared decision making. 2014 [cited 24 November 2014]. Available from: <http://www.safetyandquality.gov.au/our-work/shared-decision-making/>.
2. Birth after Previous Caesarean Birth RCOG 'Green Top' Guideline No 45: 2015
3. RANZCOG College Statement: C-Obs 38 Planned Vaginal Birth after Caesarean Section, March 2019
4. Queensland Clinical Guidelines. Normal birth. Guideline No. MN20.12-V5-R25. Queensland Health. 2020. Available from: www.health.qld.gov.au/qcgl/.
5. Royal Australian and New Zealand College of Obstetricians and Gynaecologists. Categorisation of urgency for caesarean section. C-Obs 14. College Statement. July 2019.

12. Document owner

Enquiries relating to this guideline may be directed to:

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13. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
Version 4	22/08/2023	22/08/2026	Minor amendment following clinical incident investigation

14. Authorisation

This guideline has been authorised and issued by the Director Nursing and Midwifery.

Authorised by	Helen Fullarton – Director Nursing and Midwifery
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